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/ **Practice Exam - 1**

Correct Answer

Next Q

Practice Exam - 1

Question 109 of 117



The patient is a 62-year-old man with a past medical history of asthma and Crohn's disease who presents to the ER with 2 weeks of fatigue, nausea, shortness of breath and palpitations. In the emergency room, his blood pressure is noted to be 100/62 mmHg (baseline unknown), heart rate 104 bpm and he is saturating 95% on room air. CXR demonstrates pulmonary edema and EKG demonstrates sinus tachycardia with precordial <2 mm ST depressions. Troponin is elevated and the patient is admitted. On admission, he is noted to have repeated episodes of NSVT and is initiated on amiodarone. Emergent TTE demonstrates biventricular dysfunction with an LVEF of 20% and a nondilated LV. He is initiated on diuretics and lisinopril. His troponin continues to increase and he is sent for cardiac catheterization. Coronary angiography demonstrates non obstructive CAD. LVEDP is 25 mmHg and an RHC is performed that shows an RAP of 14 mmHg, PA of 38/27 mmHg and a cardiac index of 1.7 L/min/m².

Which of the following is an appropriate next step?

- A. Initiate Solu-Medrol 1 mg daily [MRC3.1][VE3.2]
-  B. Perform a rapid respiratory panel and viral serologies
-  C. Perform emergent endomyocardial biopsy
- D. Initiate inotropes

Commentary References

Content Code of Blueprint: Specific etiology (giant cell)[MRC2.1][VE2.2]

Task: diagnosis

Teaching Point: The test taker should recognize the indications for endomyocardial biopsy in acute new onset de novo heart failure/fulminant myocarditis

Rationale:

This patient is presenting with fulminant myocarditis as evidenced by new onset cardiogenic shock, biventricular dysfunction, elevated troponins and arrhythmias. Given the recent onset of heart failure with hemodynamic compromise, this is a class I indication for endomyocardial biopsy based on a joint statement from the AHA, ACC and ESC with the more broader recommendation in 2016 AHA statement. The predominant concern prompting endomyocardial biopsy is to rule out giant cell myocarditis which while rare carries a substantially more morbid and mortal course with a substantially lower rate of transplant free survival. Early diagnosis is pivotal in management and decision making. In this patient, the patient's prior history of Crohn's disease and presentation with DT increase the pretest probability for giant cell myocarditis. The primary focus in the setting of fulminant myocarditis is stabilization and diagnosis.

Choice A is incorrect because while lymphocytic myocarditis is more common than GCM and may in fact be what this patient has, there is no data to support the routine use of corticosteroids and lymphocytic myocarditis. To RCTs have been performed that showed no clinical benefit. Despite this, steroids are often given in fulminant myocarditis presentations.

Choice B is not correct as viral serologies and rapid respiratory panels lacks sensitivity and specificity for diagnosis of viral myocarditis. Routine viral serologies are not recommended.

Choice D is incorrect as the patient is having significant arrhythmias and inotrope use can worsen these. Temporary mechanical circulatory support would be favored in the situation for stabilization and pending diagnosis and determination of next steps.

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